

Executive Summary

Opening Statement & Background

Across the United States, treatment engagement for mental health and substance use concerns continues to fall short of what national need would suggest. Many adults experience depression or heavy substance use in a given year, yet only a relatively small portion receive professional care. These gaps are not evenly distributed across demographic groups and highlight ongoing challenges in ensuring equitable access to behavioral-health services.

Purpose of the Research Study

This study used nationally representative data from the 2023 National Survey on Drug Use and Health (NSDUH) to identify which demographic, socioeconomic, and clinical factors are associated with receiving mental health treatment or substance use treatment in the past year.

Methods

To identify the strongest predictors of treatment, the study applied a Group LASSO variable-selection algorithm followed by survey-weighted logistic regression models to determine which sets of factors meaningfully contribute to predicting treatment use while accounting for the complex sampling design of NSDUH. Because the dataset is cross-sectional, the results reflect associations rather than causal relationships, but they still reveal clear national patterns in who is most likely to receive care.

Overview of Findings

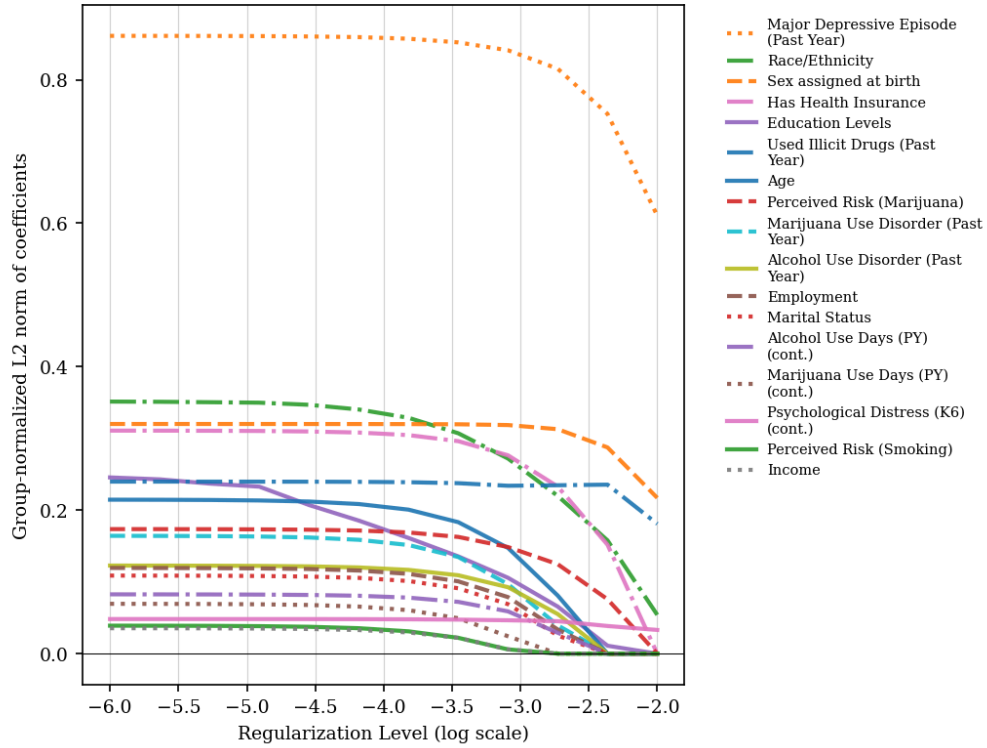
Across both treatment types, clinical need emerged as the strongest and most consistent signal. Past-year major depressive episodes were the top predictor of mental health treatment and also strongly predicted substance use treatment, with substance involvement showing similar cross-system effects. These shared predictors suggest substantial overlap in who enters each system. The Group LASSO trace plots (Figure 1-2) highlight this pattern by showing that depression remains highly influential across all model settings, reinforcing that mental health needs shape access in both service systems. At the same time, the analyses identified persistent racial and ethnic disparities. After adjusting for insurance, income, and clinical need, Black, Hispanic, Asian, and multiracial adults had significantly lower odds of receiving treatment compared to non-Hispanic White adults. Insurance coverage also played a major enabling role: having any health insurance greatly increased the likelihood of receiving both mental health and substance use care. Gender differences also emerged, with women more likely to receive mental health services and men more likely to enter substance use treatment.

Recommendations

1. Integrate mental health and substance use screening across treatment settings.
2. Expand Medicaid eligibility and strengthen behavioral-health coverage.
3. Support culturally adapted and community-centered services.

Figure 1

Group LASSO Regularization Paths for Mental Health Treatment Predictors

**Figure 2**

Group LASSO Regularization Paths for Substance Use Treatment Predictors

